

BPSS Senior Reflection: Helping people help themselves

My father enjoys telling my sister and I a quirky fable about a farmer trying to herd birds into a barn on a cold night in December. If he fails, the birds will freeze to death. The farmer's many unsuccessful tactics culminate in him yelling at the birds, frantically waving his hands and pointing. The farmer can't fathom why this isn't effective and gives up, but later notices the birds settle into the barn after a deer nestles in; they are following in the deer's example. It's a classic try, try again story, with an added theme of humanity and empathy that I love and try to emulate: to truly understand others, you must meet them on their terms, and find a way to truly listen to them.

That fable is, in many ways, the foundation of my philosophy of public service. My guiding philosophy of service is this: **service means being a tool and a guide through which people are empowered to help themselves.** The best service does not impose, rescue, or instruct from a place of authority. It listens, creates space, builds trust, and then steps back. Whether I was tutoring, sitting with a patient as a CNA in training, or medical assisting assistant at a pediatrician's, I have tried to hold onto that core conviction: I am not the answer. I am, at my best, the person who helps someone else find their own.

Tutoring

When I first started as a peer tutor at the learning center, I instinctively wanted to explain things. A student would struggle, and my impulse was to jump in with the answer, to make the confusion go away as quickly as possible. It felt helpful in the moment, but this well-intentioned instinct isn't effective in the long term.

I think of a peer tutor as Jiminy Cricket on Pinocchio's shoulder: a companion and sounding board who, crucially, fades into the background so that Pinocchio can face his challenges on his own. Students already have teachers- what they need from a peer tutor is something different: someone on their level who listens, reflects their thinking back to them, and trusts them to be, as Rademacher and Howlett put it, "naturally creative, resourceful and whole." My job is to believe in their wisdom and give them the time and space to think.

This was a lesson I have had to learn again and again. Even as a tutor for K-12 students at an organization for underserved students called Learn to Be, where the students have fewer resources, at times I have instinctively found myself trying to fill too big of a role— be the teacher, overexplain, not let them get to the answer themselves, and this absolutely hinders my students. For example, I was tutoring a 6th grader named "G" in ELA and reading comprehension, and especially towards the end of a session, or when she seems to not be getting an idea, I often desperately wanted to just tell her the answer. Yet I know it is only through teaching her the skills to figure it out herself, and encouraging her to try even when she states she "has no idea," that will allow her to grow. This is a philosophy that sometimes comes into conflict with the expectations of others: G's mother would ask me to simply tell her what unfamiliar words meant while we were reading, so she could make flashcards. It would expand her vocabulary, and I

was not opposed to doing this after the fact, but I knew her exams featured questions about deriving the meaning of words that her teachers purposefully chose because they didn't think the class would have encountered them before. I knew that if she didn't get a chance to figure out these words when we were practicing, she would struggle on these questions and not learn how to decode new words on her own. So instead, I guided her through context clues: What kind of word could go here? What do the parts of this word remind you of? What would make sense given what the rest of the sentence is saying? It was slower and sometimes frustrating, but I believed, and still believe, that it was the right way to serve her. I reached a compromise with her mother: we make flashcards at the end of the session, and she still gets to build up her decoding skills.

This experience also highlighted something uncomfortable about myself: I am not naturally confrontational, and it was difficult to hold my ground when a parent directly instructed me to do something differently. Working with G challenged me to develop a kind of quiet conviction in my approach (a flexible conviction, but a willingness to stand my ground nonetheless). Being a good guide sometimes means being willing to advocate for a method even when it isn't the path of least resistance. That is a skill I have consciously worked on, and it has made me a better tutor.

This flexible conviction has become instrumental for me: I can't impose my thoughts and goals all together, as it is just unproductive, but I need to be there to guide and help my students too. For example, I also tutored a student who I'll call "C," a tenth grader who was, without question, one of the most motivated students I had ever met. When I met her for our first session in early September, she had already identified the material she needed help with for a test not until September 27th. I was impressed, but as we got to know each other, I began to understand what was behind that drive: she was sick, and the possibility of a hospital visit was always looming at the edge of her mind. She previewed material weeks ahead because she was afraid of falling behind if she had to be away.

The complexity of working with C taught me something important about the relationship between service and empowerment. She later struggled on a chemistry test, not because she didn't understand the material (her teacher confirmed she could explain it to her classmates) but possibly because her previewing was causing her to conflate topics together. When I suggested she might try a different approach, she pushed back; not previewing felt too risky given her circumstances. Though I wanted her to guide the process of her own tutoring, I also feared her anxiety was working to her detriment. I knew if I imposed my belief that she was going astray on her, I would lose her and she would check out entirely, and simply find another tutor who allowed her to keep previewing.

I knew she had to find the answer to this problem herself— I offered my guidance, suggested compromises (why don't we focus exclusively on this content the week before the test, and make an outline of the different subjects and how they are different?). I also just let her talk about her life, her worries about her family, her worries about her academics and her future. I knew I had to take the tutor cap off and use my skills and experiences as a person to help her get through this. If I had stuck with only the tutor mentality and tried to force conventional study

strategies on her, we would not have made any progress, but by listening to her and allowing her to acknowledge her fears but also acknowledge the limitations of our current strategy, I was able to help her to find an approach she could trust. I had to hold two things at once: respect her autonomy and her hard-won coping strategies, while also gently offering the perspective of an older student who has navigated exams and anxiety before, further informing a philosophy as a tutor who helps by allowing someone to empower themselves.

Healthcare

The values I developed as a tutor grew even more when I began my clinical training as a CNA and then began working as a medical assistant. Healthcare is a setting where the power differential between provider and patient is significant, and where it is very easy for well-meaning professionals to impose their framework onto someone else's deeply personal experience. As I hope to be a (competent) physician one day, it is incredibly important to me that I build the skills now to meet people where they are at on their health journey, and not impose my own will because I "know better" and risk working to their detriment.

During my CNA clinicals, I witnessed a moment that has stayed with me. A female patient at an assisted living facility needed assistance with personal care and was refusing to let a male CNA help her. His response was one of professional offense— "didn't she know this was a reputable institution? Didn't she know we're all trained professionals?" From a technical standpoint, he wasn't wrong. But from a human standpoint, he had missed the point. She was in a vulnerable state, temporarily in that facility while recovering from surgery, and she was scared and uncomfortable. Whatever the reason for her preference— past trauma, cultural background, simple discomfort— it was hers, and in a vulnerable time, she needed all the support we could give her. I stepped in to help her, and in doing something as small as honoring that request, I believe I gave her back a small measure of agency in a situation where she had very little.

Or another time, when a long-term resident rang her call bell, voice cracking, asking for a nurse because she was in serious pain. I let the nurse know, and she said she would be there soon. I checked in about 15 minutes later, and the nurse still hadn't gotten to her. The nurse was the only one on the floor that shift, and she was just too overwhelmed and dealing with so many different requests that it was taking her a while. I knew it wasn't the same as her medication, but as a trainee CNA amongst throngs of other trainees and real CNA's, I had a moment to spare, so I offered to sit with her and try to distract her. She said that was sweet, and I spent the next hour with her as we waited for her nurse. What followed was one of the most remarkable conversations I've had. She had lived a stunning life (pursuing careers as a model, publisher, artist, and more) and now she was facing a spinal cord crisis that had landed her in that bed, waiting on a treatment plan and terrified of another surgery.

She had so many things she simply couldn't do anymore as she aged— not being able to attend her daughter's event in Paris, not being the one who takes care of others anymore— and she was worried another surgery would limit her mobility and her agency even further. "You never imagine that someone is going to have to take care of you like this," she said. I didn't have a perfect response. But I could be present, and I could let her lead the conversation

wherever she needed it to go. I asked her before I stayed whether she wanted company or privacy. She wanted company. That choice was hers, and I honored it. In a setting where so much had been taken out of her hands, that small act of asking felt significant.

As a medical assistant now, I bring this same orientation to the families I see every day at the pediatric clinic. Many of them, especially first-time parents, come in with worries that may seem minor from a clinical standpoint. But they are not minor to the people who have them. My job is to listen, to validate, and to be a reassuring presence- not to make them feel foolish for coming in. I have been working on my Spanish specifically for this reason: to be able to offer a calming voice in the same language when a parent is stressed about their child's health.

Carrying it Forward

I want to be a physician. And one of my deepest commitments as I move toward that goal is to resist the pull toward paternalism that healthcare training can encourage. Medicine has historically operated from a model in which the physician knows best and the patient complies, and this is a model that has caused real harm, particularly to people whose identities and experiences have too often been dismissed. I want to be the kind of doctor who listens first, who brings their expertise as a resource rather than a verdict, and who remembers that the person in the exam room is, at the end of the day, the foremost expert on their own life. My experiences as a service scholar have given me a framework for that, and I hope to carry forward this philosophy in my career and beyond.